

Cape Cod Hospital/Falmouth Hospital

Exceptions and Escalations Guide

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What's New?

Refer to the [Insurance P&P Updates Tracking](#) document and click the **Cape Cod-Falmouth** tab at the bottom to view recent changes.

Scope of Program

Ongoing Accounts \$100.00 to \$10,000 (list date prior to 8-1-23)

All Payors excluding

- Workers' Compensation
- Auto
- Third Party Liability
- Out-of-State Medicaid
- Out-of-Country

Med-Metrix receives accounts 30 days after the initial bill date.

Ongoing Accounts \$100.00 to \$3,999 (list date after 8-1-23)

All Payors excluding

- Auto
- Barnstable Jail,
- Health Safety Net
- Hospice
- Indian Health Services
- Oncology and Radiation Oncology service areas
- Out of Country
- Out-of-State Medicaid
- Third Party Liability
- Tufts
- Worker's Compensation

Med-Metrix receives accounts 60 days after the initial bill date.

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FACS

Allowed Contact Codes

Note: No other contact codes should be used without Supervisor/Manager approval.

- **INC:** Insurance contacted
- **OTC:** Other party contacted
- **RPC:** Responsible party contacted
- **OIC:** Online insurance contact – You must be able to find claim status paid, in process, or denied
- **IBC:** Inbound call
- **CWR:** Correspondence worked no call required
- **AWR:** Account worked no call required
- **LTM:** Left Message (voicemail or with 3rd party) for Responsible Party, Employer, Insurance Company Representative, or Doctor's office

Extended Hold Times

1. For multiple account status check - You may disconnect, if hold time exceeds 20 minutes.
 2. For single account status check - You may disconnect, if hold time exceeds 10 minutes.
 - This is specifically for accounts that do not exceed 2 previous attempts.
- Note:** Try to schedule callbacks at various times of day.
3. If you have had 2 previous attempts, you are required to hold to obtain claim status, regardless of the hold time.
 4. [Document](#) in windows **8** and **10**.

Documentation

Along with following the [Standards of Documentation](#) the information below should be included in your documentation:

- **If the claim has multiple components**, you will need to identify the date of service and **total charge of the claim in question**, not a complete breakdown. *(i.e., A breakdown of TC and DOS is not required for a single claim or single charge line or when working a project noting accounts for unposted remits.)*
 - Institutional and Pro Fee

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- Split bill
- Charges related to Clinical and Non-Clinical Trials

Take Appropriate Contact for Your Work

An individual should [only take one contact per day](#) when working an account regardless of the number of calls made or actions required to resolve the account.

Remember to take credit for your work:

1. Enter **1** in the **New Calls** field if a phone call was made or received. If **no call** required, enter **0**.
2. Enter **1** in the **New Contacts** field. (*i.e., spoke with patient or insurance, left message, checked payor website, verified EOBs*)
3. Enter the appropriate **Contact Code** in the Code field. (*i.e., AWR, IBC, INC, RPC, etc.*)

Updating Insurance Window 60

- Click here to view the [Updating Insurance in window 60](#) job aid.

2IAC

Note: The 2IAC disposition must be worked daily.

You must review notes in window 10 to determine the reason the account has been placed in a 2IAC disposition and your initial request was denied.

Remember to delete the comment in window **10** once you have resolved the issue.

Note: If you are unable to address the issue, add it to the issue log to be reviewed with your supervisor and update the disposition to 2SUP with a wait date of 7 days.

If the account was returned due to an invalid adjustment request:

1. Change the disposition and wait date based on the next actions required.
2. Take appropriate [contact](#) for your work.
3. [Document](#) in windows **8** and **10**.

Printing Request was incorrect for example:

- Tagged the wrong work queue
- Instructions were confusing

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- Rebill is not warranted
 - Required attachment or **ICN** was not provided
1. Change the disposition and wait date based on the next actions required.
 2. Take appropriate [contact](#) for your work.
 3. [Document](#) in windows **8** and **10**.

Address/Telephone Updates

If the patient's address or telephone number needs to be updated:

- Supervisor will escalate to Operations Manager via email including:
 - Patient's Name
 - Medical Record Number
 - Hospital Account #
 - FACS #
 - The new address/telephone information
- Operations Manager will escalate to Insurance Client Engagement who will then escalate to Self-Pay Client Engagement.

Adjustments/Write Offs

Special Note: Write-off / adjustments should be taken by an approved specialists.

No patient responsibility is to be NRP'd to the secondary or the patient until the appropriate adjustments are completed.

If the request **is not** valid:

1. Change the disposition in window **1** to **2IAC** with a wait date of '**T**' for today.
2. In window **650**, remove the tag in the **Adjustment Pend** field.
3. Take appropriate [contact](#) for your work.
4. [Document](#) windows **8** and **10** with a note stating why the account is being sent back to the requesting specialist.

The process below only applies to individuals who have been approved to complete contractual adjustments/write offs in Epic.

≤2,500 and documentation is present

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If the documents supporting the write off **are** in Epic **and less than or equal to \$2,500.00** and the request is valid:

- Refer to the [Adjustment Codes Training Matrix](#) for the appropriate code

In FACS

1. Change the disposition in window **1** to **2ADS** and extend the wait date **3** days.
2. In window **650**, remove the date stamp in the **Adjustment Pend** field.
3. Take appropriate [contact](#) for your work.
4. [Document](#) in windows **8** with the detailed reason for the adjustment request.

In Epic

5. From the **Liability Bucket** screen, click on the **claim** line associated with the adjustment request.
6. Click **Follow-up**.
7. The **Bucket Activity** screen will pop up on the right-hand side.
 - Click on Write-off Denial
 - Enter a note with the CPT that is being written off and the denial reason.
 - Update Denial/Remark
 - Status should be updated from write off needed to Completed.
 - Resolution Category should be updated to Denied amount Written-off
 - Under the Post Adjustment section choose the adjustment code for the denial and enter the expected reimbursement amount for the denial.
 - Under the Post Not allowed Adjustment section choose 3000 and enter the contractual adjustment amount. Then click accept.
8. In the reference # field, describe in general terms what you have done then click **Accept**.
 - i.e., "taking contractual adjustment per EOB, moving balance of \$3.00 to NRP"

Note: If more than one type of adjustment needs to occur, each will need to be posted separately regardless of the code being used.

i.e., If you need to posted a contractual, a GZ modifier line that needs to be contractualized and small balance write off, post as follows

Note: You can only use Write-Off Denial Bucket activity if you have a contractual and a write off. If you don't have both you will need to use the separate Bucket Activities for Update Denial/Remark and Post Adjustment and/or Post Not allowed Adjustment. Post adjustment is for write-off's and Post Not Allowed Adjustment is for contractual.

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Cigna

- Rev code 270
- The assigned Exception Specialist has been approved to add **HCPCS code A4649** to the claim in SSI if a HCPCS code is missing.
- If the claim **is not** in SSI, the Exception Specialist will add the account to the Billing Supervisor Review Needed Work Queue [Activity Code 306] so that the code can be added, and the claim resubmitted.

Combining/ Linking Accounts

- Follow the [Combine Accounts](#) job aid.

Insurance Updates

- Follow the [Updating Primary Insurance](#) or [Updating Secondary Insurance](#) job aid.

Missing Payments and Misapplied Payments

Confirm the payment has not posted in Epic and that all necessary information per the payment research request procedures has been obtained.

If the request is not valid:

1. Change the disposition in window **1** to **2IAC** with a wait date of '**T**' for today.
2. In window **650**, remove the tag in the **Missing Pymt** or the **Misapplied Pymt** field.
3. Take appropriate [contact](#) for your work.
4. [Document](#) in windows **8** and **10** with a note stating why the account is being sent back to the requesting specialist.

If the request is valid:

1. Change the disposition in window **1** to **2FRS** and extend the wait date **30** days.
2. [Document](#) in window **8** that the account was sent to the client for review.
3. Send a secure email to **Lisa Simonds** at lsimonds@capecodhealth.org and Susan Roberts smroberts@capecodhealth.org using the following email format and attach the necessary document(s).
 - Subject:
SECURE: Indicate Missing Payment or Misapplied Payment

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- Body of the email:

Explain reason and request for payment posting to be corrected and documents scanned.

If the client advises to w/off or adjust the charge:

- Follow the [Adjustments](#) protocol.